NCCP 00379 · VERSION 4

Your treatment, explained

NCCP 00379: adjuvant treatment of grade II glioma after radiotherapy, or palliative treatment of recurrent high-grade glioma. · NCCP 00379 v4 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

PCV chemotherapy · 42-day cycle

42-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · nccn-patient

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Treatment days → recovery and tests → review
Your 42-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Your treatment and its purpose
Taking treatment safely
Tests, urgent symptoms and follow-up
Understanding your particular glioma
Tests and scans: what each one tells you
Brain symptoms, medicines and when to call
Your overall treatment plan and choices
Living well, rehabilitation and support
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

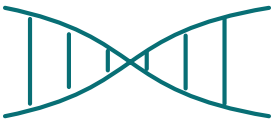
How your treatment works

PCV combines procarbazine, lomustine and vincristine. In this 42-day regimen it may follow radiotherapy for grade II glioma or be used to help control recurrent high-grade glioma. Ask which setting and treatment aim apply to you.

ORAL CHEMOTHERAPY CAPSULE

Lomustine

CCNU



Damages DNA

Lomustine damages genetic material so cells cannot divide normally. It can affect cancer cells and healthy bone-marrow cells. Blood counts may fall several weeks after the single dose, so a day without capsules is not a day without risk.

One scheduled dose → DNA damaged → Delayed blood-count effects possible

What to watch for

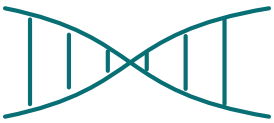
Take only the prescribed Day 1 dose. Do not take it daily or repeat it without oncology instructions. Report fever, bleeding or new breathing symptoms.

Sources: nccp

ORAL CHEMOTHERAPY CAPSULE

Procarbazine

Procarbazine capsules



Interferes with DNA and cell growth

Procarbazine is converted in the body into substances that interfere with DNA and cell growth. Healthy blood-forming and digestive tissues can also be affected. It has important interactions with some medicines, alcohol and certain foods, so a pharmacy review and written instructions are essential.

Capsules on specified days → DNA processes disrupted → Tumour and healthy cells affected

What to watch for

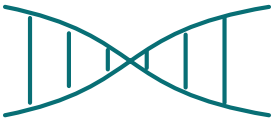
Follow the exact Days 8–21 calendar and the pharmacist's medicine, food and alcohol instructions. Report fever, rash or persistent vomiting.

Sources: nccp

CHEMOTHERAPY GIVEN INTO A VEIN

Vincristine

Vincristine infusion



Disrupts the machinery used for cell division

Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Infusion into a vein → Cell division disrupted → Nerves and bowel may be affected

What to watch for

Report tingling, weakness, walking difficulty or constipation before the next dose. Tell staff immediately if the infusion site stings, burns or swells.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Brain / energy

Glioma / PCV

Fatigue or neurological changes need review. New seizures, sudden weakness or speech difficulty need emergency assessment.

3 Blood / infection

Lomustine / procarbazine

Counts may fall weeks after lomustine and worsen over cycles. Fever, sudden illness, bruising or bleeding needs urgent advice.

5 Liver / kidneys

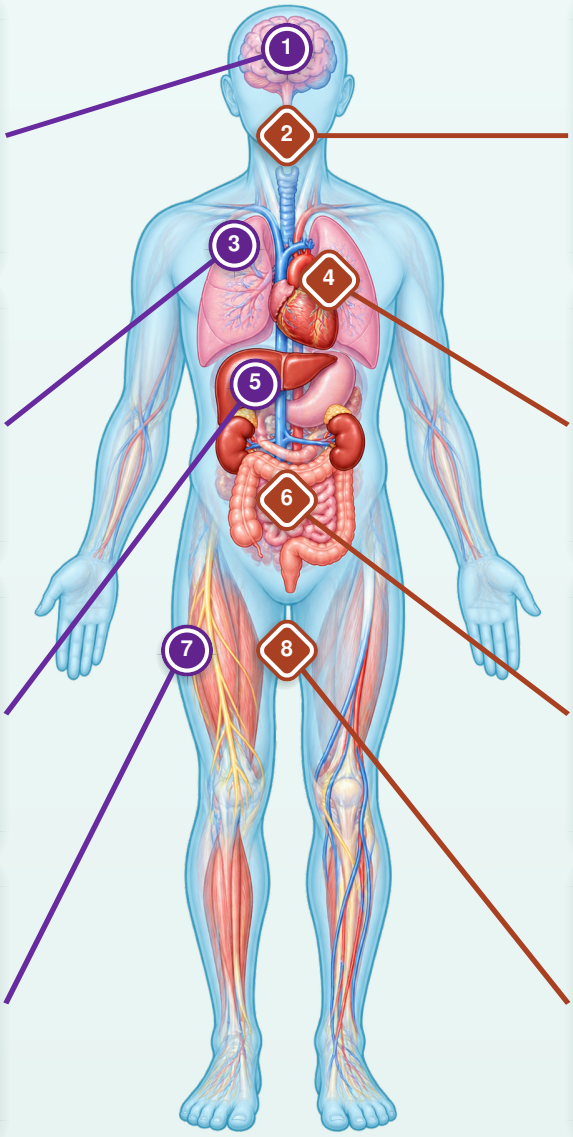
PCV

Attend organ-function blood tests. Report yellow skin or eyes, dark urine, reduced urine or dehydration rather than waiting for the next cycle.

7 Nerves / movement

Vincristine

Report tingling, numbness, weakness, jaw pain or difficulty walking before the next infusion. Symptoms can affect hands, feet or other areas.



2 Mouth / eating

PCV

Mouth soreness or appetite loss can affect intake. Keep mouth care and ask for help with pain, ulcers or difficulty eating.

4 Lungs

Lomustine

New cough or breathlessness needs assessment. Lung injury can develop with treatment; keep baseline and any repeat lung tests.

6 Bowel / nausea

PCV / vincristine

Use the nausea and preventive bowel plan. Severe constipation, abdominal pain, vomiting or inability to pass wind needs urgent advice.

8 Fertility / pregnancy

PCV

Discuss fertility preservation and contraception before starting. Ask for written precautions for patients and partners; do not breastfeed during treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Treatment days → recovery and tests → review

THREE MEDICINES ON DIFFERENT DAYS

PCV · 42-day cycle

D1 lomustine | **D8–21 procarbazine** | **D8 + D29 vincristine**

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 42 days. The number of cycles is individual; treatment continues only while benefit and tolerance support it.

DELAYED BLOOD-COUNT EFFECTS

Recovery and the next-cycle decision

Review before Day 43

The next cycle can be delayed or changed according to blood counts, organ tests, nerves, bowel symptoms and the treatment goal. A calendar or card stamp is not permission to restart.

For adjuvant treatment the source plans PCV to begin within four weeks after radiotherapy. The two PCV cycle lengths are separate regimens; do not interchange their calendars.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed MRI and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00379 v4 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · Lomustine medac current product information · January 2026

THE WHOLE TREATMENT JOURNEY

PCV chemotherapy · 42-day cycle: treatment and checks

NCCP 00379: adjuvant treatment of grade II glioma after radiotherapy, or palliative treatment of recurrent high-grade glioma.

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 42 days. The number of cycles is individual; treatment continues only while benefit and tolerance support it.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00379	FBC, renal/liver profile, glucose; baseline lung function	FBC, renal and liver tests before each treatment	Lomustine SmPC: blood count at least weekly for at least 6 weeks after each dose	Review the lowest counts and recovery before another cycle	Follow persistent blood, lung, kidney or liver effects
Lungs / bowel / peripheral nerves NCCP 00379	Baseline lung function; record bowel and nerve symptoms	Review before treatment; repeat lung testing with prolonged therapy or symptoms	Call for cough, breathlessness, severe constipation or new weakness	Check whether each PCV component remains suitable	Continue review of unresolved effects
Neurology / medicines / function NCCP 00379	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00379	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1

PCV · 42-day cycle

Three medicines on different days

PCV combines procarbazine, lomustine and vincristine. In this 42-day regimen it may follow radiotherapy for grade II glioma or be used to help control recurrent high-grade glioma. Ask which setting and treatment aim apply to you.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00379

2

Recovery and the next-cycle decision

Delayed blood-count effects

PCV combines procarbazine, lomustine and vincristine. In this 42-day regimen it may follow radiotherapy for grade II glioma or be used to help control recurrent high-grade glioma. Ask which setting and treatment aim apply to you.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00379

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.38

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 42 days. The number of cycles is individual; treatment continues only while benefit and tolerance support it.

Treatment timing: displayed days 1–42

Treatment or check	Days in this window	What happens
Lomustine: single dose	Day 1	Take once on Day 1 only, not daily.
Procarbazine daily	Days 8–21	Use the exact 14-day calendar and pharmacy interaction instructions.
Vincristine infusion	Day 8	IV treatment after review.

Treatment or check	Days in this window	What happens
Second vincristine infusion	Day 29	Review bowel and nerve symptoms before treatment.
Recovery and safety checks	Days 30–42	Counts can be low after capsule days finish.
Symptoms throughout the cycle	Days 1–42	Call for fever, bleeding, breathlessness or severe bowel symptoms.

Lomustine is a single dose: Taking lomustine daily by mistake can cause serious harm. Keep the Day 1 instruction visible and ask the pharmacist to check your understanding.

Delayed marrow effects: Lomustine can cause delayed and cumulative suppression of blood counts. Rest days still need infection and bleeding awareness, and some cycles need delay or adjustment.

Fourteen procarbazine days: Use Days 8–21 inclusive from the treatment table. Do not add Day 22 from conflicting wording elsewhere; ask pharmacy if a supplied calendar differs.

Weekly blood counts after lomustine: The current product information calls for blood counts at least weekly for at least six weeks after a dose. Keep the arranged checks even when you feel well and on days without chemotherapy.

> **Why these bands stay broad:** The cycle view shows relative treatment days, not a prediction of when symptoms will peak. Serious symptoms can occur at any time, including rest days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00379 v4 · current Irish regimen ↗

> **There are no guaranteed "safe days".** Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Complete timing and symptom notes

Day 1–7: One lomustine dose on Day 1

Take only the labelled Day 1 dose. No procarbazine is scheduled until Day 8. Check the capsule supply and anti-sickness plan; do not repeat lomustine.

Day 8–21: Daily procarbazine and first vincristine

Take procarbazine each day from Day 8 through Day 21. Attend the Day 8 vincristine infusion and required tests. Follow the bowel, alcohol, food and medicine-interaction plan.

Day 22–29: Procarbazine finished; Day 29 infusion

There are no planned procarbazine doses after Day 21. Vincristine is scheduled again on Day 29 after assessment. Report numbness, weakness or constipation before the infusion.

Day 30–42: Recovery before the next cycle

No scheduled PCV medicines in this period. Blood-count effects can still develop or persist. Attend tests and restart only after team review.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Lomustine is taken once on Day 1 only; procarbazine is taken daily on Days 8–21; vincristine is given into a vein on Days 8 and 29. Follow the pharmacy label for capsule number, food and timing. Do not open or crush capsules. Lomustine is not a daily medicine. Vincristine must only be given intravenously by trained staff.

STEP 2

Missed or vomited medicine

If a capsule dose is missed or vomited, contact the oncology pharmacy for advice. Do not take extra capsules, repeat the single lomustine dose, or extend procarbazine days without instructions.

STEP 3

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

STEP 4

Procarbazine interactions

Avoid alcohol during treatment. Ask for a written food list and medicine interaction check, including the period after procarbazine when precautions still apply. Do not assume an over-the-counter cold remedy, antidepressant, pain medicine or herbal product is safe without pharmacy review.

STEP 5

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

STEP 6

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

STEP 7

Appointments and transport

Before starting: full blood count, kidney and liver tests, glucose and baseline lung-function assessment. NCCP requires blood count, kidney and liver tests before each treatment. Low counts can be delayed and cumulative after lomustine, so keep any additional blood tests the team arranges. Repeat lung-function assessment during prolonged therapy or when clinically indicated. MRI timing follows the individual glioma plan. Current lomustine product information additionally advises weekly blood counts for at least six weeks after each dose; the team should arrange these alongside the NCCP pre-treatment checks.

STEP 8

Help with the daily plan

Ask a trusted person to help check the calendar if memory, vision, fatigue or hand strength makes treatment difficult. Confirm who is responsible for checking doses, collecting medicine, transport and calling for help. Keep consent and personal preferences central.

Sources: NCCP 00379 v4 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Nausea prevention on the correct days

Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.

2 Prevent constipation

Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.

3 Food, alcohol and medicine review

Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.

4 Support for neurological symptoms

Keep any prescribed seizure medicine and steroid plan separate from the PCV calendar. Report new weakness, tingling, gait change or constipation before vincristine. A medicine review is needed before adding any treatment for symptoms.

Sources: nccp

TREATMENT REFERENCE · 01

Your treatment and its purpose

Use the regimen and phase agreed with your team.

Why this treatment?

PCV combines procarbazine, lomustine and vincristine. In this 42-day regimen it may follow radiotherapy for grade II glioma or be used to help control recurrent high-grade glioma. Ask which setting and treatment aim apply to you.

Sources: nccp

What each part does

Lomustine damages genetic material so cells cannot divide normally. It can affect cancer cells and healthy bone-marrow cells. Blood counts may fall several weeks after the single dose, so a day without capsules is not a day without risk. Procarbazine is converted in the body into substances that interfere with DNA and cell growth. Healthy blood-forming and digestive tissues can also be affected. It has important interactions with some medicines, alcohol and certain foods, so a pharmacy review and written instructions are essential. Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Sources: nccp

Schedule and planned duration

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 42 days. The number of cycles is individual; treatment continues only while benefit and tolerance support it.

Sources: nccp

Evidence and uncertainty

Adding PCV after radiotherapy improved long-term survival in that study population. RTOG 9802 used an eight-week PCV schedule; it does not provide a schedule-specific efficacy estimate for this six-week regimen. Its results must not be presented as the expected benefit for recurrent high-grade glioma. No single survival figure is assigned to this card. The study used historical pathology classification; applicability requires specialist review of the current integrated diagnosis.

Sources: rtog9802

TREATMENT REFERENCE · 02

Taking treatment safely

The exact prescription and calendar matter.

Taking the treatment

Lomustine is taken once on Day 1 only; procarbazine is taken daily on Days 8–21; vincristine is given into a vein on Days 8 and 29. Follow the pharmacy label for capsule number, food and timing. Do not open or crush capsules. Lomustine is not a daily medicine. Vincristine must only be given intravenously by trained staff.

Sources: nccp

Missed or vomited medicine

If a capsule dose is missed or vomited, contact the oncology pharmacy for advice. Do not take extra capsules, repeat the single lomustine dose, or extend procarbazine days without instructions.

Sources: nccp

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

Sources: nccp

Procarbazine interactions

Avoid alcohol during treatment. Ask for a written food list and medicine interaction check, including the period after procarbazine when precautions still apply. Do not assume an over-the-counter cold remedy, antidepressant, pain medicine or herbal product is safe without pharmacy review.

Sources: nccp

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

Sources: nccp

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

Sources: nccp

Appointments and transport

Before starting: full blood count, kidney and liver tests, glucose and baseline lung-function assessment. NCCP requires blood count, kidney and liver tests before each treatment. Low counts can be delayed and cumulative after lomustine, so keep any additional blood tests the team arranges. Repeat lung-function assessment during prolonged therapy or when clinically indicated. MRI timing follows the individual glioma plan. Current lomustine product information additionally advises weekly blood counts for at least six weeks after each dose; the team should arrange these alongside the NCCP pre-treatment checks.

Sources: nccp · lomustine-smpc

Help with the daily plan

Ask a trusted person to help check the calendar if memory, vision, fatigue or hand strength makes treatment difficult. Confirm who is responsible for checking doses, collecting medicine, transport and calling for help. Keep consent and personal preferences central.

Sources: nccn-patient

TREATMENT REFERENCE · 03

Tests, urgent symptoms and follow-up

Keep planned tests and know how to get help.

Your monitoring plan

Before starting: full blood count, kidney and liver tests, glucose and baseline lung-function assessment. NCCP requires blood count, kidney and liver tests before each treatment. Low counts can be delayed and cumulative after lomustine, so keep any additional blood tests the team arranges. Repeat lung-function assessment during prolonged therapy or when clinically indicated. MRI timing follows the individual glioma plan. Current lomustine product information additionally advises weekly blood counts for at least six weeks after each dose; the team should arrange these alongside the NCCP pre-treatment checks.

Sources: nccp · lomustine-smpc

When to seek help

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

Sources: nccp · nccn-patient

Support medicines

Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan. Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice. Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them. Keep any prescribed seizure medicine and steroid plan separate from the PCV calendar. Report new weakness, tingling, gait change or constipation before vincristine. A medicine review is needed before adding any treatment for symptoms.

Sources: nccp · nccn-patient

A delay is a treatment decision

Blood counts, organ tests or symptoms may mean a dose is held, reduced or stopped. This is a safety decision, not a reason to take extra capsules later. Ask which medicines change, which support medicines continue and when the next assessment is due.

Sources: nccp

After the final dose

Agree follow-up blood tests, symptom review, MRI timing and who to contact. Continue to report new problems; effects can outlast chemotherapy. Keep your medicine list and written plan current.

Sources: nccp

TREATMENT REFERENCE · 04

Understanding your particular glioma

Your diagnosis includes the tumour's cells, molecular findings and behaviour, not just the word brain cancer.

The name and location

Gliomas begin in supporting cells of the brain or spinal cord. Astrocytoma, oligodendroglioma and glioblastoma are different diagnoses. A tumour that spread to the brain from another organ is a brain metastasis and is not covered by these glioma regimens. Record the exact integrated diagnosis and where it is located.

Sources: nccn-patient

Grade is not the same as stage

Grade describes features linked to how a tumour behaves. Adult gliomas are usually discussed by their type, grade and molecular findings rather than the stage system used for many other cancers. Grade 4 does not mean cancer has spread through the body. Ask your specialist to explain your own report and any older terminology in the treatment protocol.

Sources: nccn-patient · nci-cns

IDH and 1p/19q

IDH1 or IDH2 changes and loss of parts of chromosomes 1p and 19q help classify particular gliomas. The combination of findings matters: one result should not be read in isolation. These tests are usually performed on tumour tissue. They do not automatically mean a change has been inherited or that relatives need testing.

Sources: nccn-patient

MGMT promoter methylation

MGMT is involved in repairing certain DNA damage. Methylation of its promoter can reduce that repair activity and helps the team discuss the likely benefit of temozolomide in relevant settings. It is not a blood tumour marker, a percentage of tumour left, or an individual survival prediction. Record the reported result and ask how it affects your treatment choice.

Sources: nccn-patient

A useful diagnosis record

Keep the biopsy or operation date, pathology wording, grade, IDH result, 1p/19q result and MGMT result if tested. Mark a result as pending or not applicable rather than assuming it is negative. Ask whether specialist pathology review or further testing would change the plan.

Sources: nccn-patient

TREATMENT REFERENCE · 05

Tests and scans: what each one tells you

Safety blood tests, tumour tissue tests and MRI answer different questions.

Safety blood tests

A full blood count checks infection-fighting white cells, oxygen-carrying red cells and platelets that help stop bleeding. Kidney and liver tests help assess treatment safety. Attend the regimen's required tests even when you feel well; a reassuring scan does not replace them. The schedule in this guide separates checks before, during and after treatment.

Sources: nccp · nccn-patient

MRI and neurological review

MRI is the main scan used to assess many gliomas. Your team also considers symptoms, strength, movement, speech, memory, daily function and medicines such as steroids. Tell staff before MRI about implants or metal, kidney problems, previous contrast reactions or difficulty lying still. Ask about support if scans make you anxious.

Sources: nccn-patient

Changes on a scan need interpretation

After treatment, inflammation and other treatment effects can resemble tumour growth on MRI. This is sometimes called pseudoprogression. The team may need comparison scans, specialist imaging or further assessment before deciding what a change means. New or worsening symptoms still need prompt review; do not assume they are harmless treatment effects.

Sources: nccn-patient

An individual follow-up timetable

Write the next MRI date, what question it should answer and who will explain the result. Timing depends on your diagnosis, treatment phase and previous findings. General follow-up intervals in an international booklet are not automatically your schedule. These regimens do not require a generic CT chest/abdomen/pelvis or a blood tumour marker before every cycle.

Sources: nccn-patient

Keep results with decisions

Use the passport to distinguish a test being booked, completed and reviewed. Record the agreed next step and whom to call if an appointment or result is missing. Do not alter chemotherapy or steroids yourself in response to a number or scan report.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Write down your usual headache, strength, walking, speech, vision, memory, mood, sleep and seizure pattern before treatment. Note new changes, when they started and how they affect daily tasks. With your permission, a trusted person can help spot changes or record what happened.

Sources: nccn-patient

Urgent neurological changes

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Sources: nccp · nccn-patient

Steroids need a separate written plan

Steroids may be prescribed to reduce swelling around the tumour. They are not automatically needed for everyone. Ask for the exact dose, time, review date and any taper plan. Do not stop or change them suddenly without advice. Report sleep or mood changes, increased thirst, infection symptoms or muscle weakness and ask which additional checks or protection you need.

Sources: nccn-patient · nhs-dexamethasone

Seizure medicines and rescue instructions

If prescribed, take seizure medicines on the agreed schedule. Ask what to do after a missed or vomited dose and whom to contact for breakthrough symptoms. Some medicines can affect mood, alertness or interact with treatment. Do not start, stop or switch them yourself. A rescue medicine needs an individual plan and training for whoever may give it.

Sources: nccn-patient

Safety at home and outside

Ask your clinical team about driving, work, bathing, swimming, heights and machinery in light of your symptoms and any seizures. Local medical and licensing rules need individual advice; this guide does not give a universal driving clearance date. Occupational therapy can help adapt daily routines and the home.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Surgery or biopsy, radiotherapy, chemotherapy and follow-up have different jobs. Ask your team to draw the intended sequence and identify where you are now. Concurrent treatment means treatments overlap; adjuvant treatment follows the main treatment. A change of phase may change the dose, blood tests and support medicines.

Sources: nccn-patient

Agree what treatment is trying to achieve

Ask whether the aim is to control the tumour, delay growth, relieve symptoms, preserve function or a combination of these. Discuss likely benefits, burdens, alternatives and what would prompt a pause or stop. The answer depends on the exact diagnosis, previous treatment and your priorities.

Sources: nccn-patient

Put evidence in context

Clinical trials describe groups of people and may use older pathology names or different schedules. They cannot predict one person’s response or lifespan. Ask how closely the evidence matches your tumour and treatment. No benefit number from one schedule should automatically be applied to another.

Sources: nccn-patient

Discuss options available to you

Ask about appropriate clinical trials, specialist opinions, symptom support and other treatment options. A medicine or device described in an international guide is not automatically licensed, funded or suitable in Ireland. The treating multidisciplinary team explains what is available and relevant.

Sources: nccn-patient

Make room for your priorities

Bring up fatigue, thinking, independence, family time, work, travel and side effects you most want to avoid. Ask for information at a pace and in a format that suits you. You may ask for a trusted person to join the discussion, written notes and time to reconsider decisions.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about word-finding, attention, memory, reading, vision or planning difficulties. Rehabilitation may include physiotherapy, occupational therapy, speech and language therapy, or neuropsychological support. Written routines, rest breaks and help with medicine organisation can make day-to-day care more manageable.

Sources: nccn-patient

Food, nausea, bowels and energy

Report poor appetite, weight change, vomiting or constipation early. Ask for dietitian help if eating is difficult. Use the prescribed anti-sickness and bowel plan; do not wait until symptoms are severe. Choose activity that is safe for your balance and strength and balance it with rest. Ask about treatable causes of fatigue.

Sources: nccp · nccn-patient

Emotional support and carers

Anxiety, low mood, changes in behaviour and uncertainty can affect both patients and carers. Tell the team what has changed and ask for appropriate psychological or social support. Carers may need their own information, practical help and breaks. Agree who the team may share information with.

Sources: nccn-patient

Work, transport and finances

Ask the specialist nurse or social worker about getting to treatment, time off work, benefits, caring responsibilities and support at home. Plan who can help with daily capsules and appointments if your memory, vision or fatigue makes these difficult.

Sources: nccn-patient

Supportive care and future wishes

Supportive and palliative care can help with symptoms and quality of life while active treatment continues. You can discuss future care preferences and who you want involved in decisions at a time that suits you. Ask the team for Irish advice and documentation rather than using legal or hospice rules from a US booklet.

Sources: nccn-patient

Information that works on paper

Keep the printed schedule, current medicine list, tests, contact numbers and emergency instructions together. This handbook can be read in sequence without internet access. The wallet card and optional QR code help others find the regimen information; they do not replace your prescription or emergency plan.

Sources: nccn-patient

PCV evidence depends on the treatment setting

RTOG 9802 compared radiotherapy alone with radiotherapy followed by PCV in selected patients with grade II glioma.

Adding PCV after radiotherapy improved long-term survival in that study population.

Sources: rtog9802

PREPARE FOR THE CONVERSATION

Questions worth asking

What is my exact diagnosis, grade and relevant molecular result?

Ask for the integrated pathology wording and an explanation of IDH, 1p/19q and MGMT where relevant. The diagnosis and treatment history guide how this regimen applies to you.

Which phase and capsule calendar am I on now?

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 42 days. The number of cycles is individual; treatment continues only while benefit and tolerance support it.

Can you check the Day 1 lomustine and Days 8–21 procarbazine instructions?

Ask the pharmacist to mark each medicine on the calendar. Lomustine is a single Day 1 dose; procarbazine is a different medicine taken over fourteen days. Confirm food, alcohol, interaction and bowel instructions.

Which tests must happen before I can take the next dose?

Before starting: full blood count, kidney and liver tests, glucose and baseline lung-function assessment. NCCP requires blood count, kidney and liver tests before each treatment. Low counts can be delayed and cumulative after lomustine, so keep any additional blood tests the team arranges. Repeat lung-function assessment during prolonged therapy or when clinically indicated. MRI timing follows the individual glioma plan. Current lomustine product information additionally advises weekly blood counts for at least six weeks after each dose; the team should arrange these alongside the NCCP pre-treatment checks.

Who helps with seizures, steroids, memory or daily tasks?

Ask for a named contact, written seizure and steroid plans if prescribed, and rehabilitation or practical support. Agree who can help you organise treatment.

Which symptoms mean contact the team today?

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

When will we review the benefit and my priorities?

Ask for the next clinical and MRI review, how treatment effects will be distinguished from growth, and what would lead to continuing, changing or stopping treatment.

> **Try explaining the plan back:** Can you show the days for each medicine, the current phase, your next blood tests and whom to call for fever, vomiting or new neurological symptoms?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00379 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00379 v4 · current Irish regimen
- NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference
- PCV evidence depends on the treatment setting
- Lomustine medac current product information · January 2026
- NCI: adult central nervous system tumours · diagnosis and grade education
- NHS: dexamethasone · use and stopping safely

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6971/379_V4_PCV.pdf

https://www.nccn.org/patients/guidelines/content/PDF/brain-gliomas-patient.pdf

https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/common_SPCs/Lomustine_medac-spc-common.pdf

https://pubmed.ncbi.nlm.nih.gov/27050206/

https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq

https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/